

Sample #

Gender: Male

DOB:

Race: White

Report Date:

Pathology Report:

ICD-0.3
Carcinoma, urothelial NOS
8/20/13
Site C67.0
Bladder, trigone
path
Overlapping lesion of
bladder C67.8
JED 7/22/13

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Left pelvic lymph nodes; dissection:

- Metastatic urothelial carcinoma in a matted lymph node (1/1).
- The tumor focus measures 4.0 cm.
- Tumor extensively involves perinodal adipose tissue and lymphovascular spaces.

B. Left pelvic lymph nodes; dissection:

- Metastatic urothelial carcinoma in four of four lymph nodes (4/4).
- Tumor present in perinodal adipose tissue and lymphovascular spaces.
- Segment of vas deferens, no tumor present.

C. Right pelvic lymph nodes; dissection:

- Metastatic urothelial carcinoma in five of nine lymph nodes (5/9).
- Tumor extensively involves perinodal adipose tissue and lymphovascular spaces.

D. Bladder and prostate; cystoprostatectomy:

- Urothelial carcinoma of the urinary bladder, high grade, transmurally invasive into the perivesical and the left perivesicular adipose tissue.
- Urothelial carcinoma in situ.
- Extensive lymphovascular invasion present.
- Soft tissue margin of resection positive for carcinoma.
- Margins of resection (prostatic urethra, right and left ureter), no tumor present.
- See pathologic parameters.
- Prostate with extensive necrotizing granulomas consistent with BCG therapy effect.
- High grade prostatic intraepithelial neoplasia (PIN).

E. Left distal ureter; excision:

- Segment of ureter, no tumor present.

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TCGA-FD-A6TH-01A-PR

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F. Left "peri urethral" tissue; excision:
- Fibroadipose tissue, no tumor present.

G. Right distal ureter; excision:
- Segment of ureter, no tumor present.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Flat urothelial carcinoma
 2. Grade of tumor: High grade
 3. Depth of invasion: Extravesical adipose tissue
 4. Tumor distribution: Solitary 5.8 x 3 x 2.5 cm
- Dome
Right lateral wall
Left lateral wall
Trigone
Posterior wall
5. Ureteral margins: Negative
 6. Distal urethral margin: Negative
 7. Soft tissue margin or serosa: Positive
 8. Lymph nodes: Metastatic urothelial carcinoma in ten of fourteen lymph nodes (10/14)
 9. pTNM: pT3N2

Effective January 1, 2010 this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow [], MD
Electronically Signed Out by [] M.D

Clinical History:

The patient is a year-old male with urothelial carcinoma undergoing a cystoprostatectomy with lymph node dissection.

Specimens Received:

- A: Left pelvic lymphnode
- B: Left pelvic lymphnode
- C: Right pelvic lymphnode
- D: Bladder and prostate
- E: Left distal ureteral stitch non margin
- F: Left peri urethral tissue
- G: Right distal ureter stitch non margin

Gross Description:

The specimens are received in seven containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "left pelvic lymph node". Received fresh for intraoperative consultation is a 4 x 3.5 x 3.5 cm aggregate of matted, firm, fibroadipose tissue adherent to a lymph node measuring 4 x 3.5 x 1.7 cm. A representative section of the lymph node is frozen and read as, "metastatic urothelial carcinoma" per Dr. []. The remaining lymph node is serially sectioned and entirely submitted as A2-A12. The remaining fibroadipose tissue submitted as A13. The specimen is entirely submitted.

B. The second container is additionally identified as, "left pelvic lymph node". Received fresh and placed in formalin is a 5.5 x 4.2 x 1 cm aggregate of yellow, lobulated fibroadipose tissue. The specimen is dissected to reveal for lymph node candidates ranging from 0.3-3 cm in greatest dimension. The specimen is entirely submitted as follows

- B1: 2 lymph node candidates
- B2: 1 lymph node candidate
- B3-B4: 1 lymph node candidate, serially sectioned
- B5-B9: Remaining fibroadipose tissue

C. The third container is additionally identified as, "right pelvic lymph node". Received fresh and placed in formalin is a 6 x 4 x 1.5 cm aggregate of soft, yellow, lobulated adipose tissue. The specimen is dissected to reveal 7 lymph node candidates ranging from 0.3-3.7 cm in greatest dimension. The specimen is entirely submitted as follows

- C1: 3 lymph node candidates
- C2-C4: 1 lymph node candidate, serially sectioned
- C5: 1 lymph node candidate, serially sectioned
- C6-C9: 1 lymph node candidate, serially sectioned
- C10-C12: 1 lymph node candidate, serially sectioned
- C13-C14: Remaining fibroadipose tissue

D. The fourth container is additionally identified as, "bladder and prostate". Received fresh and placed in formalin is a 365.96 g, 19 x 11.5 x 5 cm

cystoprostatectomy specimen consisting of a 6.5 x 5.5 x 3 cm bladder with attached mesenteric fat and a 5.3 x 3.5 x 3.5 cm prostate. The right seminal vesicles measure 2.0 x 2.5 x 1.5 cm and right vas deferens measures 4 x 0.5 cm. The left seminal vesicles measures 2.0 x 2.5 x 1.3 cm and left vas deferens measures 4 x 0.5 cm. The right ureteral stump measures 2 x 0.4 cm, the left measures 0.4 x 0.3 cm, and each demonstrates an intact, patent lumen. The urethral stump measures 0.6 x 0.6 cm.

The right half of the prostate and bladder is inked blue and the left half is inked black. The bladder and prostate are opened anteriorly along the urethra. The outer surface of the prostate is reddish tan and shaggy. Sectioning demonstrates diffuse white-tan, ill-defined lesions, which cluster together throughout the entire prostate. The ill-defined white lesions abut black ink. A gross photograph is taken.

The opened bladder reveals a 3 x 2.2 cm yellow-tan ulcer with a hyperemic border in the left posterior and left lateral wall. On cut section, the mass measures 5.8 x 3 x 2.5 cm and extends into and through the muscularis propria into the perivesical fat, abutting the lateral left margin and 0.1 cm from the deep margin. Gross photographs are taken. The surrounding bladder mucosa is wrinkled, pink-tan with a uniform 1.3 cm wall thickness. Bilateral ureteral orifices, adjacent to the trigone, are identified and probe patent.

Representative sections are submitted as follows:

- D1: Distal prostatic urethral margin
- D2: Right ureter resection margin
- D3: Left ureter resection margin
- D4-D5: Prostate apex
- D6-D8: Right prostate (apex to base)
- D9-D11: Left prostate (apex to base)
- D12-D13: Additional right side of prostate (near base)
- D14: Representative right seminal vesicles and vas deferens
- D15: Representative left seminal vesicles and vas deferens
- D16: Right lateral wall
- D17: Left lateral wall
- D18: Trigone
- D19: Anterior wall
- D20: Posterior wall
- D21: Dome
- D22: Mass extending into pericystic fat, abutting lateral margin
- D23: Mass closest to deep margin
- D24-D26: Additional sections of mass
- D27: Mass w/adjacent normal bladder parenchyma

E. The fifth container is additionally identified as, "left distal ureter stitch non-margin". Received fresh and placed in formalin is a 0.3 cm portion

of ureter with a diameter of 0.3 cm oriented with a stitch on the non-margin side. The opposite side is inked black and submitted en face E1.

F. The sixth container is additionally identified as, "left periureteral tissue ". Received fresh and placed in formalin is an unoriented fragment of yellow-tan soft tissue measuring 1.4 x 0.8 x 0.3 cm. The specimen is submitted in toto as F1.

G. The seventh container is additionally identified as, "right distal ureter stitch on margin". Received fresh in place in formalin is a 1.3 cm portion of ureter with a diameter of 0.3 cm and oriented with a stitch marking the non-margin side. The opposite side is inked black and submitted en face G1.

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
IPAA Discrepancy		✓
Prior Malignancy History	✓	
Dual/Synchronous Primary Cancers	✓	
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	AMX	Date Reviewed: 7/9/13